

Horizon 2 — Beyond the Compliant Digital Thread

MDR Devices Medizintechnik GmbH — where the same thread takes you next, and why it changes what compliance costs you

To	Executive Board, MDR Devices Medizintechnik GmbH
From	much. Consulting
Re	The Horizon 2 Roadmap — companion to the Compliant Digital Thread proposal

Why we are showing you this now

Everything in our main proposal — the credential gate, the quarantine automation, the serialized traceability thread, the analytic spine — is **Horizon 1**: the validated foundation that makes MDR Devices compliant by structure rather than by memory. This document exists because we believe a partner should be selected not only for the system they build, but for what they see *after* it. The three moves below all run on data the Compliant Digital Thread already captures — no new data entry, no second foundation. That is the point: **built once, the thread keeps paying**. One of these moves carries a statutory deadline that applies to MDR Devices regardless of any decision you take on this proposal.

Horizon 1 makes you compliant. Horizon 2 makes compliance pay — the same thread files your registrations, prints your implant cards, and answers your auditors.

Move 1 — The EUDAMED Autopilot

STATUTORY DEADLINE · 27 NOVEMBER 2026

The regulatory position, stated plainly: EUDAMED's first four modules — including **UDI/Device registration** — have been mandatory since **28 May 2026**. New devices must be registered before being placed on the EU market; devices already on the market must be registered by **27 November 2026**. This obligation sits with MDR Devices today, and across the industry it is largely being met with spreadsheets and manual portal entry — slow, error-prone, and unrepeatable.

What we propose: the thread already holds every field the UDI/Device module demands — Device Identifier from the product master, Production Identifier structure, manufacturer and device attributes. The Autopilot generates EUDAMED-conformant registration datasets directly from those records, maintains a submission queue with acknowledgement tracking (submitted / acknowledged / rejected, each state audit-trailed), executes the legacy-device bulk registration as a managed service ahead of the November deadline, and keeps registrations synchronized when device data changes — because EUDAMED is a living register, not a one-time filing.

THE USP

Your EUDAMED submission stops being a project and becomes a **by-product of master data you already maintain**. The deadline is met once; the synchronization is met forever. No competing approach we are aware of derives the registration directly from the same validated record that drives traceability and recall.

EUDAMED UDI/Device Registration — Autopilot

DEVICE / FAMILY	UDI-DI	STATUS	
SN-HC-0001 · HipCore Ti-64	04012345678901	Acknowledged	
SN-HC-0002 · HipCore Ti-64	04012345678901	Acknowledged	
SN-HC-0003 · HipCore Ti-64	04012345678901	Queued	
Legacy — HC-Gen1 family (2,140 devices)	040123456778**	Bulk prepared	batch 3 of 6
Legacy — SpineFix family (860 devices)	040123456821**	Data gaps: 14 records	remediation queue

🕒 LEGACY DEADLINE — 27 NOV 2026

147 days remaining. Mandatory since 28 May 2026 — every device placed on the EU market must be registered. This queue is generated from data the thread already holds.

Your EUDAMED obligation exists whether or not you buy anything — the only question is whether you type it by hand. The thread already holds every field the UDI/Device module demands.

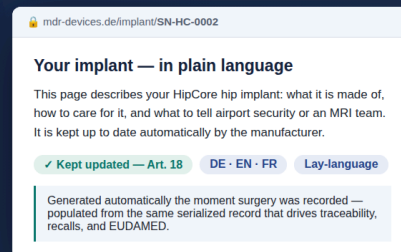
The regulatory position: as a manufacturer of implantable orthopedic devices, MDR Devices must supply an **implant card** with each device — carrying device name, serial, UDI and manufacturer details — and must keep **patient-accessible, lay-language device information updated on its website**, in the patient's own language. Most manufacturers meet the card obligation manually and the website obligation poorly. Both are usually invisible to the people they exist for.

What we propose: the moment your field team records *Surgery Complete* — the same consumption event that already drives revenue recognition in Horizon 1 — the system automatically **(1)** generates the print-ready Article 18 implant card from the serial's UDI record, zero transcription; **(2)** publishes the patient's device page on your own portal — plain language, multilingual, and *updated automatically* whenever the device record changes; **(3)** prints a QR code on the card linking to that page, so a patient — or an emergency clinician anywhere in Europe — reaches the device record in seconds.

THE USP

Compliance becomes **visible to your customers for the first time**. Hospitals and surgeons see a manufacturer whose implants arrive with a living digital passport; procurement committees remember it. The thread stops ending at your invoice — **it ends in the patient's pocket**. That sentence is the difference between compliance as overhead and compliance as product.

HORIZON 2 · MOVE 2 — PATIENT IMPLANT PASSPORT



The thread doesn't end at the invoice. It ends in the patient's pocket — an Article 18 implant card and a living device page, printed and published automatically by the consumption event.

The platform position: Odoo 19 embeds AI agents natively — trained on your own records and documents, able to query live data and draft outputs inside the same permission model as every other user. This is not a bolt-on chatbot; it is a platform capability your license already includes.

What we propose: once the thread has accumulated operating history, a scoped agent serves three jobs: **natural-language audit retrieval** ("show me every lot that breached its storage envelope in the last twelve months, and where those devices are now" — answered from live records, sources linked); **drafted deviation and CAPA summaries** for your quality team's review; and a **post-market surveillance trend watch** across returns, complaints and reprocessing data — the Article 83–86 obligation that grows with your device fleet.

THE GOVERNANCE LINE — OUR USP IS RESTRAINT

The copilot drafts; it never releases. Every AI output lands behind the same two-person release gate as every other control in this system, and the agent's scope, training corpus and logs sit inside the validated boundary, documented like any other Category 5 component. AI accelerates your compliance team. It is never itself the approver — and a partner who tells you otherwise is selling you an audit finding.

MDR Devices Medizintechnik GmbH

Inventory

Sales

Accounting

Quality

Manufacturing

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AI / Compliance Copilot

HORIZON 2 · MOVE 3 — COMPLIANCE COPILOT

Compliance Copilot

Odoo 19 native AI agent · RAG on your own records

Auditor: "Show me every lot that breached its storage envelope in the last 12 months, and where those devices are now."

Copilot: 2 lots recorded storage-condition deviations in the period.

- LOT-2549-C — excursion 9.1 °C for 40 min (envelope 2–8 °C), 12 Jan 2026. Disposition: Extended Hold → released after CoA review. 14 devices consumed, 6 in WH/Stock.
- LOT-2601-F — logger gap 3 h, 02 Mar 2026. Disposition: Scrap (8 units) — [records linked](#).

Sources: 2 quality check records · 3 stock lot histories · SOP-QA-011 §4. Draft summary created for QA review.

✓ Grounded in live records

Drafts only — human releases

Agent scope & logs inside the validated boundary

AI accelerates the compliance team; it is never itself the approver. The copilot drafts — every output lands behind the same two-person gate as everything else in this system.

The roadmap — three waves, each self-justifying

WAVE	DELIVERABLE	WHY THEN · COMMERCIAL SHAPE
Now → Nov 2026	EUDAMED Autopilot + legacy bulk registration	Statutory deadline 27 November 2026 — the obligation exists regardless; the only variable is manual effort. Fixed-fee module plus a one-time managed service.
At go-live	Patient Implant Passport	Rides the consumption event the moment Horizon 1 creates it. Per-device-family configuration plus a recurring passport service.
Post-stabilization	Compliance Copilot	Needs 6–12 months of thread data to be worth querying. Subscription — compliance capability as a running service, not a project.

What this roadmap says about the partnership you are choosing

We could have ended our proposal at the system you asked for. We are showing you Horizon 2 because we think the selection you are really making is not between ERP configurations — it is between a vendor who closes a ticket and a partner who is already thinking about your November deadline, your patients' pockets, and what your auditors will ask in two years. Every move above runs on the foundation in our main proposal, which is also the honest commercial statement: **the discovery sprint you approve today is the cheapest decision on this entire roadmap** — everything after it compounds on the same thread, built once.

Scope notes, stated with the same candour as the main proposal: each Horizon 2 move is extension-layer work, validated at GAMP Category 5 on its own track; the implant-card layout follows MDCG 2019-8 and per-Member-State language requirements, confirmed during its design phase; EUDAMED interface specifics are sized against the Commission's current machine-to-machine specification at the time of build. Nothing on this page is claimed as out-of-the-box — that discipline is the same one that runs through everything else we have shown you.